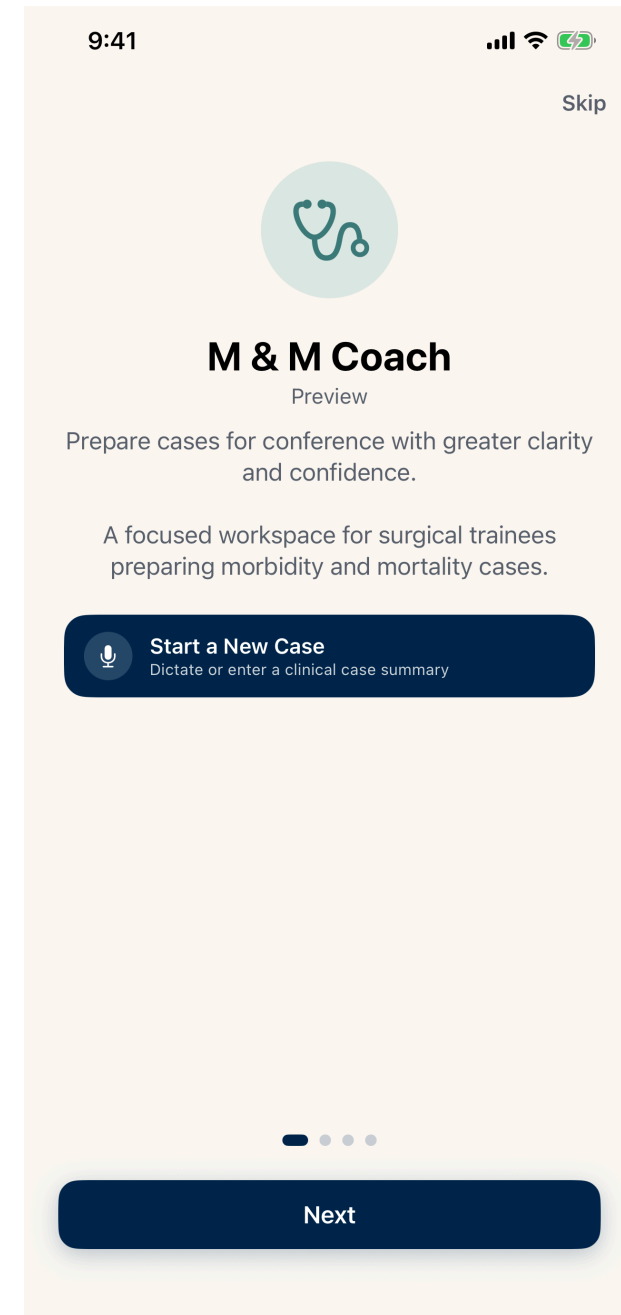


Reviewer Instructions

M & M Coach

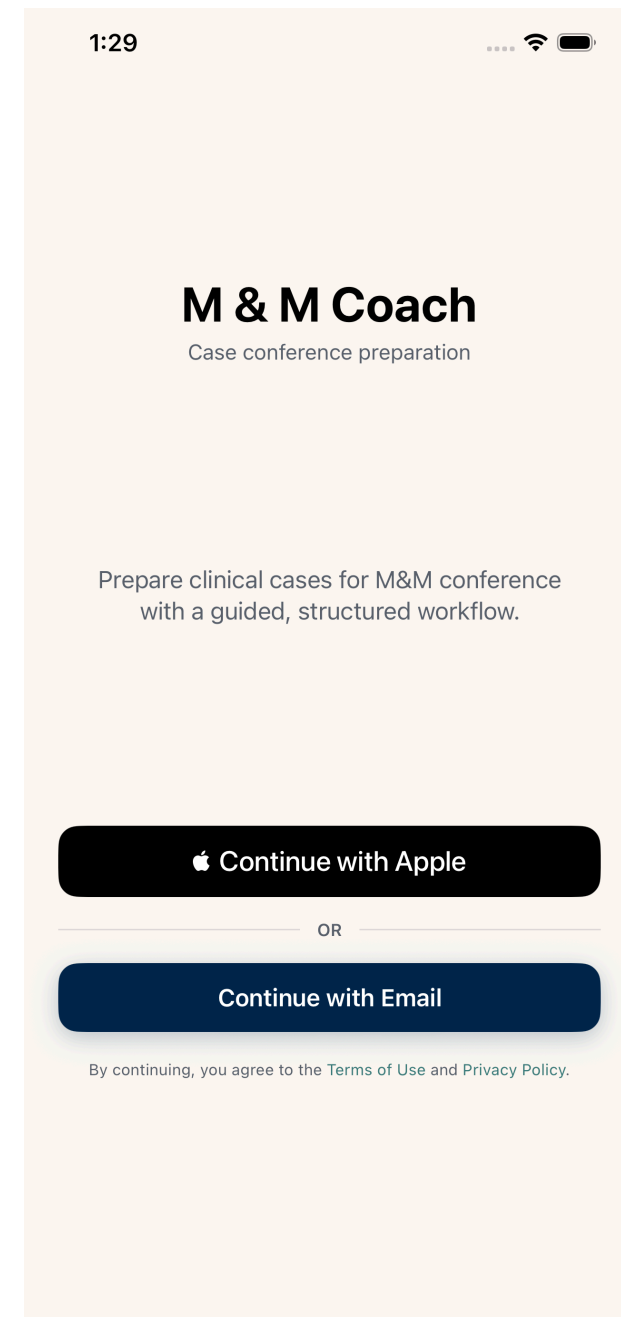
Onboarding

- First launch shows 4-page intro
- Skip or Get Started leads to sign-in



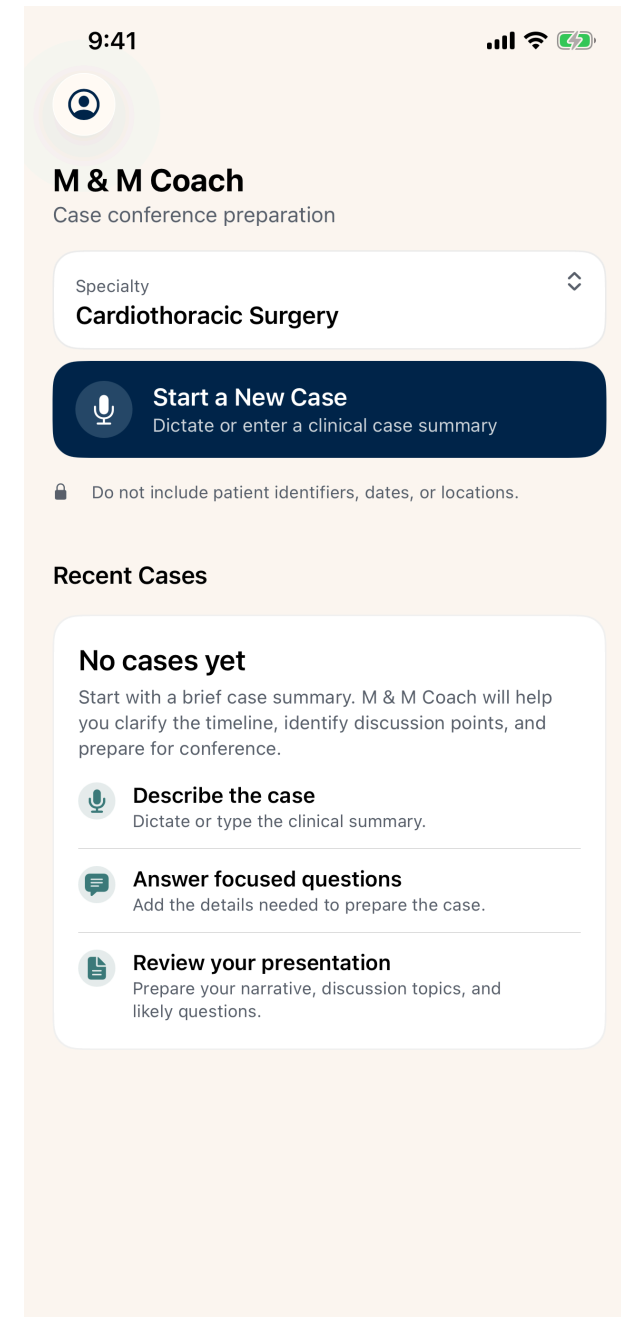
Sign-in

- Can sign in or create new account
- Use Apple sign-in or email/password
- No email verification required



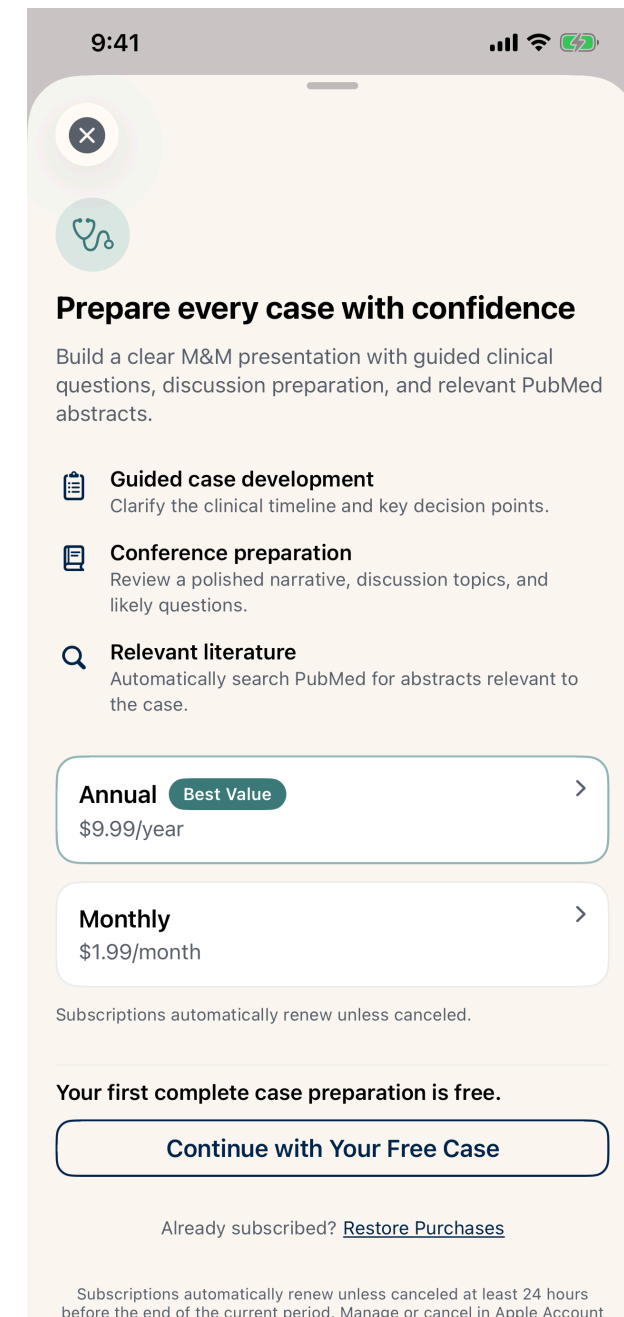
Home Screen

- Pick your specialty - leave on default
- Tap **Start a New Case**



Paywall

- All accounts get one free case preparation
- This option is not available after saving first case
- Two auto-renewing subscription options
 - Annual
 - Monthly



Case Description

- User dictates, pastes, or types text into box
- Tap **Continue**
- On-device check flags likely patient identifiers before submission
- Not exhaustive - always use de-identified or fictional case details

The screenshot shows a mobile app interface for entering a new case. At the top, the status bar shows the time 2:06, signal strength, and battery level. Below the status bar is a navigation bar with a back arrow on the left and the title "New Case" on the right. The main content area is titled "Describe the case" and includes a subtitle: "Present it as you would at M&M conference. Do not include protected health information, including patient names, hospital or institution names, or dates of service." Below this is a text input area with a prompt: "Dictate or type -- whichever is easier." The input area contains two paragraphs of text. The first paragraph describes a 68-year-old man with multivessel coronary artery disease who underwent elective coronary artery bypass grafting with three grafts. The second paragraph describes the post-operative course, noting that the operation was initially uncomplicated but the patient developed hypotension and required vasopressor support. At the bottom of the input area is a "Continue" button. Below the input area is a "Done" button.

2:06

< New Case

Describe the case
Present it as you would at M&M conference. Do not include protected health information, including patient names, hospital or institution names, or dates of service.

Dictate or type -- whichever is easier.

A 68-year-old man with multivessel coronary artery disease underwent elective coronary artery bypass grafting x3: left internal mammary artery to the left anterior descending artery, saphenous vein graft to the obtuse marginal branch, and saphenous vein graft to the right coronary artery.

The operation was initially uncomplicated, and he was transferred to the ICU hemodynamically stable. Several hours after arrival, chest tube output became increasingly sanguineous. The patient developed hypotension and required vasopressor support. His hemoglobin also decreased, and he received blood products.

Because chest tube drainage remained persistently elevated with ongoing hemodynamic instability, the decision was

Continue

Done

Case Details

- If more information is required, user will see this view
- This view may repeat several times depending on response

12:00

< Preparing Your Case

I need one more detail:

What were the findings during the mediastinal re-exploration?

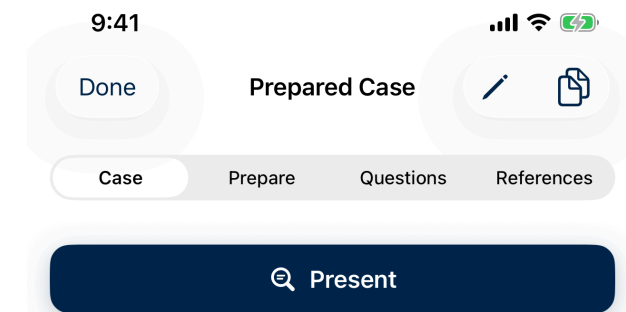
Answer as you would explain it out loud...

Tap to dictate

Submit Answer

Prepared Case

- Narrative summary of user supplied case
- User may edit this text or copy it
- Tapping **Present** button will show a view with large font so trainee can more easily read the case aloud.



A 68-year-old man with multivessel coronary artery disease underwent elective coronary artery bypass grafting ×3, which included a left internal mammary artery to the left anterior descending artery, a saphenous vein graft to the obtuse marginal branch, and a saphenous vein graft to the right coronary artery. The operation proceeded without complications, and he was transferred to the ICU in a hemodynamically stable condition. Approximately two hours after his arrival in the ICU, the chest tube output became increasingly sanguineous. Over the next three hours, the drainage totaled approximately 900 mL. Concurrently, the patient's blood pressure declined, necessitating the initiation of vasopressor support. His hemoglobin level decreased to 7.1 g/dL despite transfusion with packed red blood cells, fresh frozen plasma, and platelets. Due to persistent chest tube drainage and hemodynamic instability, the patient was taken back to the operating room approximately five hours after the initial procedure. During re-exploration, active bleeding was identified from a branch near the saphenous vein graft to the obtuse marginal artery. Hemostasis was achieved, the re-exploration was terminated, and the chest tube

Prepared Case

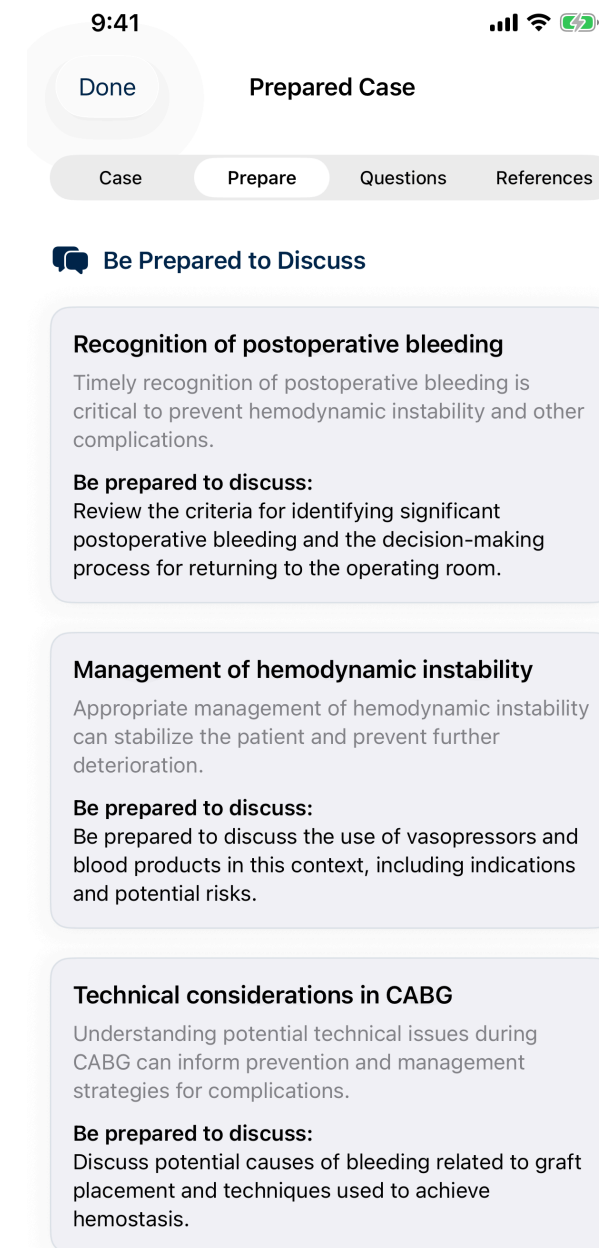
- Tapping **Present** button will show a view with large font so trainee can more easily read the case aloud.



A 68-year-old man with multivessel coronary artery disease underwent elective coronary artery bypass grafting $\times 3$, which included a left internal mammary artery to the left anterior descending artery, a saphenous vein graft to the obtuse marginal branch, and a saphenous vein graft to the right coronary artery. The operation proceeded without complications, and he was transferred to the ICU in a hemodynamically stable condition. Approximately two hours after his arrival in the

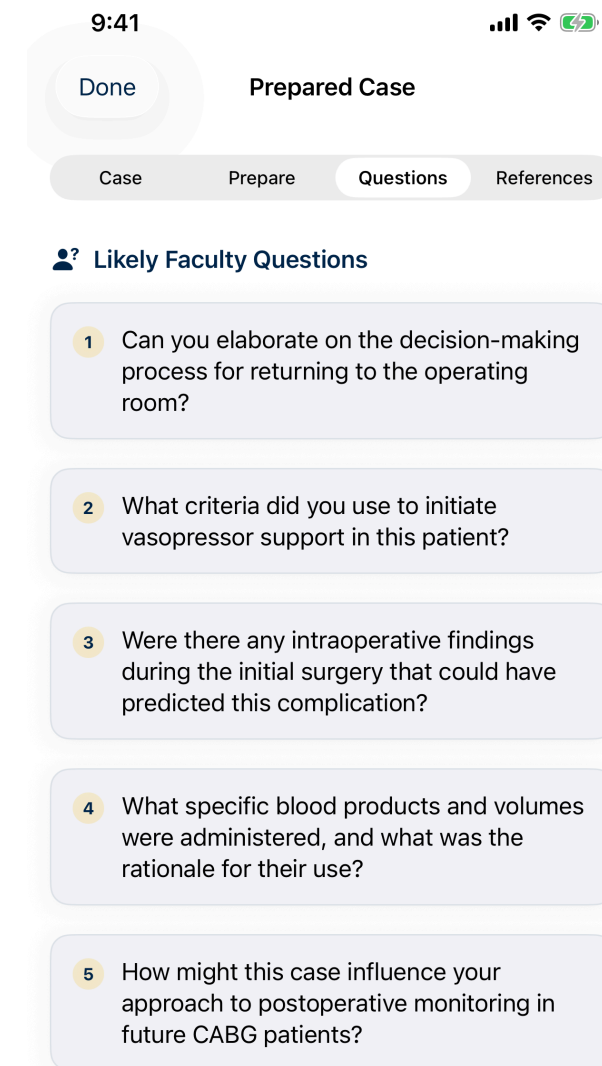
Prepared Case

- These are topics germane to the case
- This can be used to prepare trainee for discussion topics



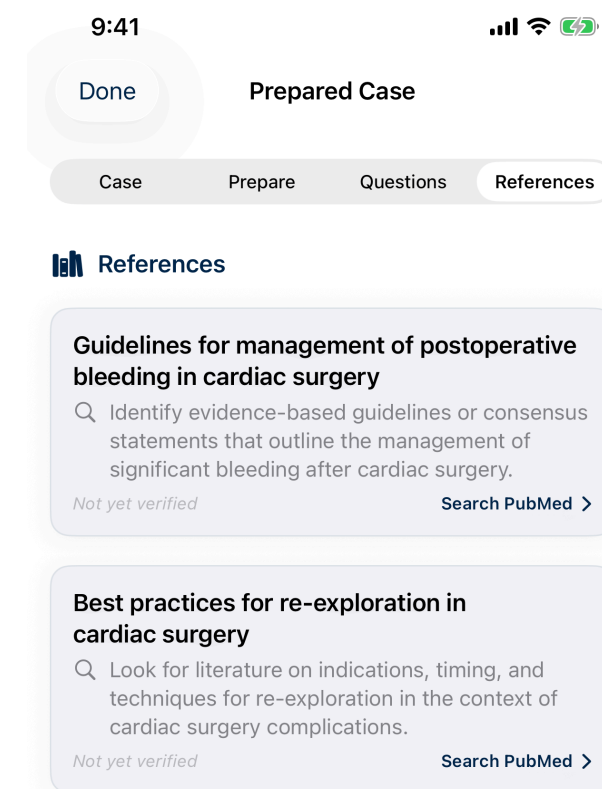
Prepared Case

- Trainee is always asked questions by faculty members
- This is a list of likely questions



Prepared Case

- These are literature search topics that apply to prepared case
- Tapping each option will search PubMed database for appropriate journal articles



M & M Coach

- This is an educational tool, not medical advice/HIPAA-compliant
- For additional information:
 - support@benderapps.dev